

1. Administrative & Study Identification

Full Study Title: An Open-Label Phase 1 Study to Evaluate PF-08046032 as Monotherapy and Part of Combination Therapy in Participants With Advanced Malignancies **Short Title/Acronym:** PF-08046032 Phase 1 Study in Advanced Cancers **Protocol Number:** PF-08046032-01 **NCT Number:** NCT06870487 **Sponsor Name:** Pfizer **Investigational Product:** PF-08046032 (CD25V Directed Antibody Drug Conjugate) and Sasanlimab (Anti-PD-1 Antibody) **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer Oncology Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and initial anti-tumor activity (Objective Response Rate) of PF-08046032 administered as monotherapy and in combination with sasanlimab in participants with advanced malignancies. **Secondary Objectives:** To evaluate progression-free survival (PFS), overall survival (OS), pharmacokinetics (PK: Cmax, AUClast, Tmax), and the immunogenicity (incidence of Anti-Drug Antibodies [ADA]) of PF-08046032 alone and in combination with sasanlimab.

2.2 Background & Rationale To address the unmet medical need for patients with advanced or metastatic lymphomas and solid tumors (e.g., NSCLC, HNSCC, melanoma) whose disease has worsened on or after standard treatments, including immune checkpoint inhibitors. PF-08046032 represents an innovative therapeutic approach designed to deplete intratumoral regulatory T cells (Tregs), thereby overcoming tumor-driven immune suppression, especially when combined with the PD-1 inhibitor sasanlimab.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose Escalation) and Combination Cohorts (Open-label) **Blinding:** Open-label **Randomization:** N/A (Sequential assignment into Parts A, B, and C)

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 100-150 evaluable patients required for dose escalation and expansion cohorts **Number of Sites:** Global, including sites in the United States and Spain **Estimated Study Start:** March 2025 **Estimated Primary Completion:** Approximately 3 Years (Up to 2028)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histological or cytological diagnosis of metastatic or unresectable malignancy. 2. Part A1: Participants with lymphomas (cHL, PTCL, large B-cell lymphoma) who have progressed on/after standard therapies. 3. Part A2/B: Participants with solid tumors (NSCLC, HNSCC, melanoma, or other limited tumor types) who have either progressed on or following prior immune checkpoint inhibitor therapy, or who have not received prior immune checkpoint therapy.

4.2 Exclusion Criteria 1. Ongoing peripheral neuropathy. 2. History of significant immune-mediated adverse event considered related to prior immune-modulatory therapy. 3. Known or suspected active autoimmune disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Part A (Monotherapy):** PF-08046032 administered at escalating dose levels on a 28-day cycle. * **Part B & C (Combination):** PF-08046032 combined with sasanlimab on a 28-day cycle. **Route of Administration:** PF-08046032 via Intravenous (IV) infusion; Sasanlimab via Subcutaneous (SC) injection. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or subject withdrawal (up to approximately 3 years).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications for symptom management (e.g., anti-emetics, analgesics). **Prohibited:** Concurrent use of other investigational agents, standard antineoplastic therapies, or immunosuppressive therapy/systemic corticosteroids that could interfere with the immunomodulatory effects of the study drugs.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Scans
Baseline	Day 0 (Cycle 1 Day 1)	First Dose of Study Drug(s), Baseline PK/ADA blood draws
Intra-cycle (Cycles 1 & 2)	Day 8, 15	Heightened frequency safety and PK (Cmax, AUClast, Tmax) evaluations
Interim	Cycles 1-4, 6, 8, then Q4 Cycles	PK (Cmax, AUClast, Tmax) and ADA evaluations pre/post-dosing, Safety Labs, Efficacy Assessment (RECIST v1.1 / Lugano) performed every 8 weeks for the first year, then every 12 weeks thereafter
End of Study	+30 days post-last dose	Final Vital Signs, Exit Interview, IP Accountability, Long-term survival follow-up initiation

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Assessment of Dose-Limiting Toxicities (DLTs) to determine maximum tolerated dose (MTD) and Objective Response Rate (ORR). **Measurement Method:** Tumor response/progression assessed by Investigator according to RECIST v1.1 (for solid tumors) and the Lugano Classification (for lymphomas).

7.2 Secondary & Exploratory Endpoints * Progression-Free Survival (PFS): Time from first dose to first documented radiographic progression or death (approx. 3 years). * Overall Survival (OS): Time from first dose to death due to any cause. * Pharmacokinetics (PK): T_{max}, C_{max}, and AUC_{last} of PF-08046032 alone and in combination. * Immunogenicity: Incidence of Anti-Drug Antibodies (ADA).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring during clinical visits and infusion days; specific focus on immune-mediated adverse events and infusion-related reactions. **Serious Adverse Events (SAEs):** To be reported within 24 hours of investigator awareness, particularly focusing on events resulting in death, life-threatening scenarios, or inpatient hospitalization. **Data Monitoring Committee (DMC):** Internal Dose Escalation Committee (DEC) to review safety data and make dose-escalation or cohort-expansion decisions between study cycles.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: * Safety Set: All participants who receive at least one dose of the investigational product. * Efficacy/Evaluable Set: Participants with measurable disease at baseline who have ≥ 1 post-baseline efficacy assessment. **Sample Size Justification:** Sample size is not driven by strict statistical power calculations but rather by standard Phase 1 clinical trial designs (e.g., 3+3 or mTPI design) to robustly evaluate safety, PK, and identify the recommended Phase 2 dose (RP2D). **Handling of Missing Data:** Efficacy endpoints like PFS and OS will use Kaplan-Meier methodology, with patients lacking progression or death events censored at the date of their last evaluable disease assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Scheduled onsite and remote monitoring visits by

Pfizer or an assigned Contract Research Organization (CRO) to ensure protocol adherence. **Audit Trail:** Robust electronic Case Report Form (eCRF) tracking with data clarification forms, ensuring secure and traceable clinical data management.

11. Study Identifiers & Governance

Primary ID: PF-08046032-01 **Registry Number:** NCT06870487 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study to Learn About the Study Medicine Called PF-08046032 in People With Advanced Cancers **Official Regulatory Title:** An Open-Label Phase 1 Study to Evaluate PF-08046032 as Monotherapy and Part of Combination Therapy in Participants With Advanced Malignancies

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Cancers (Lymphomas and Solid Tumors including NSCLC, HNSCC, Melanoma) **Diagnosis Threshold:** Metastatic or unresectable malignancy with progression on/after standard therapies **Medical Methodology:** Targeted Treg depletion and immune checkpoint inhibition (Anti-PD-1) **Optimization Target:** Maximum Tolerated Dose (MTD) identification and Objective Response Confirmation

13. Study Procedures & Methodology

Management Pathway: Stepwise dose-escalation followed by combination safety evaluation and cohort expansion. **Education Protocol (HCP):** Site Initiation Visits (SIV) detailing the management of ADC-related toxicities and immune-related AEs (irAEs). **Education Protocol (Patient):** Comprehensive informed consent process regarding novel immunotherapies, SC/IV administration, and required frequent PK sampling. **Quality Audit Mechanism:** Source Data Verification (SDV) for primary safety/efficacy endpoints, IP accountability, and continuous Pharmacovigilance signal detection.

14. Observation & Follow-Up Schedule

Total Duration: Approximately 3 Years (duration of PFS/OS measurement period) **Onsite Visit Cadence:** Every 28 days (per cycle), with heightened frequency (Day 1, 8, 15) during Cycles 1 and 2 for PK/safety **Remote Monitoring Frequency:** Inter-cycle outreach as dictated by patient safety needs. **Checklist Review Interval:** Tumor assessments generally performed every 8 weeks for the first year, then every 12 weeks thereafter.

15. Sign-off & Approval

	Role		Name		Signature		Date			:---		:---		:---		:---	
	Principal Investigator		[Insert Name]		_____		03-Apr-2026										
	Clinical Operations Lead		[Insert Name]		_____		03-Apr-2026										
	Lead Statistician		[Insert Name]		_____		03-Apr-2026										